SLEEP-06 | Therapist only - all age groups

Maintaining support and assessing ongoing difficulty

1 | Purpose, fit and boundaries

Purpose of this unit

Summarize helpful adjustments, choose minimal recording if needed, and prepare questions about an ongoing difficulty. Continuing the series depends on needs and assessment, not an obligation to finish six steps.

Choice and adaptation

Choose this sheet following professional assessment and with consent. The person may skip questions, answer aloud, or choose another tool. This is proposed original material for review; it provides no score, diagnosis, or promise of an outcome.

Boundaries specific to this unit

This is not a full CBT-I protocol, a diagnostic tool, or a requirement before seeing a doctor. Do not reduce sleep time, change treatment, or wait to finish monitoring when assessment is needed. Stop and reconsider monitoring that increases pressure or repeated checking.

Reusing existing material

Summarize only useful information from SLEEP-01–05. There is no need to complete every sheet again; medical assessment and a dedicated treatment plan remain the responsibility of the appropriate professional.

A fictional example for discussion

Fictional example: David finds a noise adjustment comfortable, but still falls asleep unintentionally during the day. With his therapist, he prepares a brief description and questions for the doctor without waiting to finish every sheet in the series.

> Draft for administrator review. Clinical review and final rights approval are still pending.

Mindful Path

SLEEP-06 | Therapist only - all age groups

Working together with the worksheet

2 | A conversation about one situation

Opening

Choose a situation the person wants to explore. It need not be the hardest situation. Check that the question is understood and the wording fits the person; their account comes before the therapist's interpretation.

1 | Question for discussion

What have I learned about adjustments, limitations, and support?

2 | Question for discussion

What ongoing difficulties or changes matter to describe?

3 | Question for discussion

What minimal information is needed for the planned assessment?

4 | Question for discussion

What questions will I ask the appropriate professional?

5 | Question for discussion

What will remain flexible until further assessment?

Closing

Summarize what became clearer and what remains open. Use the existing SES-S01 and SES-S02 for the session framework. There is no need to repeat an assessment or plan already recorded.

Possible difficulties

When recording becomes constant checking, stop it. If accessing a suitable service is difficult, explore access barriers and practical assistance instead of suggesting more monitoring.

Mindful Path

SLEEP-06 | Therapist only - all age groups

Adapting for four age groups

3 | Age is a starting point, not an assumption about ability

CH | Children, approximately ages 6 to 11

An adult manages seeking help and medical assessment, while the child describes things in their own words. Do not give the child nighttime monitoring tasks or responsibility for arranging help.

Offer one question at a time with little writing. Adult support must not replace the child's own voice.

TE | Adolescents, approximately ages 12 to 17

Agree on what the adolescent wants to describe independently and what to share with an adult. Privacy is not a promise to hide information needed for their safety.

Agree in advance what will be shared and with whom, and explain the privacy limits in this setting. Do not promise absolute confidentiality.

AD | Adults

Questions can cover functioning, conditions, and assessment options without drawing diagnostic conclusions. Medication information goes to the treating professional; changing treatment is not part of this sheet.

OA | Older adults

A verbal summary may replace a diary. A companion helps by choice and does not replace the person's own account or consent.

Offer reading support, time, and conversation according to preference. Choose help from another person together with the participant, respecting their privacy.

> Each age version needs its own review. Approval of one version does not automatically approve the others.

Mindful Path

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Practice, review and next steps

4 | Shared choice and learning from the attempt

Before offering practice sheet P

If recording is chosen, agree with the therapist in advance on minimal information and an endpoint. Preparing just one question is enough; a diary is not mandatory and need not be completed before asking for help.

At the next session

Summarize what helps and what needs further assessment. Finishing the series does not mean the difficulty is resolved, and there is no need to continue sheets that do not serve the person.

Professional and technical records

Each version has a separate review record covering age suitability, originality, rights, and fictional filling. Reviewer and date fields remain blank until assigned. File checks or a code merge are not clinical approval. Clinical information stays only in the authorized setting, outside the code repository.

Background references

The following links were used for background only. Questions and adaptations were written afresh for the expansion plan; no questionnaire, scale, or protocol was copied. Adult sources do not validate the child versions. This material makes no claim about session count or treatment effectiveness.

www.nhs.uk/conditions/insomnia/
www.nhs.uk/conditions/excessive-daytime-sleepiness-hypersomnia/

Mindful Path